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Reader: a check-writer who will argue with the strategy.

Tunza — investor draft

The stake

eCHIS does not order which sick child goes first. The care path can then fail at the decision, destination, facility, travel, acknowledgement, or returned-result step. Tunza has to exist as an attempt to solve that gap. The demo does not do this yet. [1]

Those failures are different costs of the same interval: a clinical decision that was wrong; a referral to the wrong place; a facility that could not provide the service; a patient who could not travel; a referral never acknowledged; a result that never came back to the community. [2]

A score in a database changes nothing until a named person acts. [3]

Whether that wait kills a child is unknown; no checked source documents that death. What is documented is the country's toll: [WHO Kenya 2025, printed page 60](#), estimates 355 maternal deaths per 100,000 live births, from preventable causes including postpartum haemorrhage and hypertensive disorders. Kenya's Ministry of Health names referral fragmentation and ties its developing referral policy to maternal and child health outcomes. Those are the country's numbers. They are not Tunza's results, not a market-size claim, and this page still has no lives-saved number. [4][11][12]

Kenya's Ministry of Health is developing a national referral policy on continuity, interoperability, and accountability. The country already supports [more than 107,000 Community Health Promoters](#), per the Ministry of Health — and none of that scale decides which sick child moves first. [13] That is public context, not a partnership, not an endorsement, and not evidence that Tunza implements the policy. The first decision is still unordered. Reference only: <https://health.go.ke/kenya-moves-strengthen-patient-referral-system-through-new-national-policy> [5]

Terms

Instrument: _____

Amount: _____

Discount or rate: _____

What the earliest partner may join: _____

What they do not get: _____

Proposed use of a check

These are proposals for discussion, not Evan's commitments or work already secured. There is no amount, discount, or deadline on this page. [6]

- **P1 — County introduction.** Seek an introduction in one county.
- **P2 — Data rights.** Prepare a page setting out the data rights that would be needed.
- **P3 — Shadow period.** Propose a period alongside normal care where patients are not the test and the demo does not direct their care.

This proposal is not funding a trained model or a national rollout. [6]

What exists now

The public demo has household, community health worker, and facility views, with English and Kiswahili copy. Its demonstration rules produce a next-step category: go now, get care today, monitor at home, or ask for one more answer. [7]

It is not a doctor. It is not a diagnosis. The rules are not clinically validated. Cases stay in the browser. The referral path uses fake demo facilities. It is not evidence of a working connection to a receiving clinic. It is not a substitute for emergency services or professional medical care. [8]

The public question-and-next-step demo is not the eCHIS ordering product. That ordering is designed. It is not built. [9]

Strategy questions still unanswered

- **Q1 — Label.** What outcome belongs in the label column, who records it, and when?
- **Q2 — Data rights.** What permission would allow the intended data use, including any training?

- **Q3 — One product or two.** Are we building case prioritization for eCHIS, a household question service, or two separate products?
- **Q4 — Precision floor.** How many false urgent alerts can a community health worker tolerate before they stop opening the app?

These remain questions for Evan, not decisions made by this draft. [10]

Sources

[1] fact-sheet-urgent.md S1; comparison-note.md supported line. “Has to exist” is intent, not a measured effect. Demo limit: fact-sheet-urgent.md honest product boundary. [2] fact-sheet-urgent.md S2–S7 (/Users/evanmotovich/code/Tunza/README.md:51-56). [3] fact-sheet-urgent.md S12. [4] fact-sheet-urgent.md U1–U3. Death causality remains UNKNOWN. [5] fact-sheet-urgent.md Kenya policy context. MoH URL is a reference link, not an endorsement. [6] ../tunza-vision-2026-09-23/fact-sheet.md:75. Terms remain empty under that fact sheet’s financing fields. [7] ../tunza-vision-2026-09-23/fact-sheet.md:22-25,31-32. [8] ../tunza-vision-2026-09-23/fact-sheet.md:31-39. [9] ../tunza-vision-2026-09-23/fact-sheet.md:72-74. [10] ../tunza-vision-2026-09-23/fact-sheet.md:81-85. [11] ../tunza-vision-2026-09-24-newdoc/fact-sheet-newdoc.md S13 (WHO Kenya Annual Report 2025, printed p. 60). [12] ../tunza-vision-2026-09-24-newdoc/fact-sheet-newdoc.md S14 (MoH referral-policy page, Mar 2026). [13] ../tunza-vision-2026-09-24-newdoc/fact-sheet-newdoc.md S15 (MoH node 2374, May 2026).

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